

OREGON STATE HOSPITAL

POLICY

SECTION 7: Patient Rights **POLICY: 7.012**

SUBJECT: Patient Responsibilities and
Family Responsibilities

POINT PERSON: Director of Ombuds and Family
Services

APPROVED: Sara Walker, MD

DATE: JULY 25, 2024


Interim Superintendent

SELECT ONE: ☐ New policy ☐ Minor/technical revision of
existing policy
☒ Reaffirmation of existing policy ☐ Major revision of existing policy

I. PURPOSE AND APPLICABILITY

- A. Oregon State Hospital (OSH) recognizes that health care delivery is enhanced by the involvement of the patient, appropriate to the patient's skill level, as a partner in the health care process. To facilitate the process, OSH can expect reasonable and responsible behavior and involvement on the part of a patient and the patient's family members. This policy outlines the expected responsibilities of patients, families, and OSH staff's communication of these responsibilities.
- B. This policy applies to all staff.

II. POLICY

- A. OSH will provide education about patient responsibilities and family responsibilities to a patient, the patient's guardian, and the patient's designated family in accordance with OSH Policy and Procedure 6.028, "Education for Patients," and this policy. Such education must include the patient and family's role in helping to facilitate safe care to the patient.
- B. OSH will provide resources or programs to support collaborative patient communication and problem-solving resources while the patient resides at OSH.
- C. Staff will inform a patient and guardian of the patient's responsibilities.
 - 1. Staff must review and discuss patient responsibilities with the patient during the admission process and as needed thereafter.

2. The review must be documented on the "Patient Responsibilities" form.
 - a. The patient and guardian are requested to sign the form.
 - b. If the patient is unable to sign the form, staff must note the information on the form and authenticate the form.
 - c. The completed form must be filed in the patient's medical record.
 - d. A copy must be given to the patient and guardian.

D. Patient responsibilities include, but are not limited to the following:

1. providing accurate and complete information about present symptoms, past illnesses and hospitalizations, medications, and other matters related to the patient's health to the best of the patient's knowledge;
2. reporting perceived risks in the provision of care, unexpected changes in the patient's condition, and providing feedback about care needs and expectations, which may include but is not limited to completing a Mental Health Advance Directive;
3. communicating clinical concerns or questions in person to staff, as clinical concerns communicated by any other means (e.g., email, text, voicemail) are not assured of being received;
4. asking questions when the patient does not understand what the patient has been told about care or treatment, or asking about expectations regarding follow-up care or treatment;
5. following instructions and the treatment care plan (TCP) developed in collaboration with the interdisciplinary treatment team (IDT).
 - a. The patient is encouraged to express any concerns they have about their ability to follow the TCP.
 - b. Every reasonable effort consistent with best practices will be made to modify a patient's TCP to the patient's desires and needs.
6. accepting that there may be natural outcomes if the patient is unable to follow the TCP or cues given by staff;
7. following rules and regulations concerning patient care, safety and security, and conduct;
8. showing respect and consideration toward other patients, staff, and visitors, helping control noise and environmental disturbances, and respecting other patient and OSH property; and

9. meeting financial commitments as able.
- E. Staff will provide information on family responsibilities listed below to family. Family responsibilities will be made available via:
1. Ombuds and Family Services.
 2. the OSH website,
 3. the Family Guidebook,
 4. Ombuds and Family Services information boards in campus reception areas, and
 5. Patient Financial Services (financial information only).
- F. OSH requests family enhancement and supports their involvement as a partner in the health care process by:
1. providing accurate and complete information about present symptoms, past illnesses and hospitalizations, medications, and other matters related to the patient's health to the best of the person's knowledge;
 2. reporting perceived risks in the provision of care, unexpected changes in the patient's condition, and providing feedback about care needs and expectations;
 3. communicating clinical concerns or questions;
 4. asking questions when family does not understand what has been communicated about care or treatment, or asking about expectations regarding follow-up care or treatment;
 5. expressing any concerns about the patient's ability to follow the TCP;
 6. accepting natural outcomes if the family does not follow instructions given by staff;
- G. OSH expects family to support patients in the health care process by:
1. following rules and regulations including, but not limited to: patient care, safety, security, contraband, visitation, and conduct;
 2. showing respect and consideration toward patients, staff, and other visitors, including being aware of patient needs when interacting, helping control noise and environmental disturbances, and respecting patient and OSH property;
 3. meeting financial commitments when acting as fiduciary for patient's finances;

4. assisting in maintaining health of patients, staff, and visitors by:
 - a. getting recommended vaccinations (e.g., flu)
 - b. arranging visits when physically well; and
 - c. arranging visits when emotionally calm.
- H. OSH follows all applicable regulations, including federal and state statutes and rules; Oregon Department of Administrative Services (DAS), Shared Services, and Oregon Health Authority (OHA) policies; and relevant accreditation standards. Such regulations supersede the provisions of this policy unless this policy is more restrictive.
- I. Staff who fail to comply with this policy or related policy attachments or protocols may be subject to disciplinary action, up to and including dismissal.

III. DEFINITIONS

- A. "Family" in this policy means a person or persons who play a significant role in an individual's life. A family is a group of two or more persons united by blood or adoptive, marital, domestic partnership, or other legal ties. The family may also be a person or persons not legally related to the individual (such as a significant other, friend, or caregiver) whom the individual personally considers to be family. A family member may be the surrogate decision-maker if authorized to make care decisions for the individual should he or she lose decision-making capacity or choose to delegate decision making to another.
- B. "Staff" includes employees, volunteers, trainees, interns, contractors, vendors, and other state employees assigned to work at Oregon State Hospital (OSH).
- C. "Guardian" has the same meaning as the definition in OSH Policy 6.044, "Guardianship and Conservatorship".

IV. RELATED OSH POLICIES AND PROTOCOLS

- 6.058 Admissions
- 8.044 Contraband and prohibited items
- 6.028 Education for patients
- 6.044 Guardianship and conservatorship
- 6.011 Interdisciplinary treatment team
- 7.005 Patient rights
- 7.006 Patient grievances

7.008 Patient abuse or mistreatment allegation reporting

8.026 Visitors to patients

V. REFERENCES

DHS OHA 060-041

Joint Commission Resources, Inc. (2024). *The joint commission comprehensive accreditation manual for hospitals*, RI 02.01.01. Oakbrook Terrace, IL: Author.

Oregon Administrative Rules §§ 309-106-0000 — 309-106-0020